THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

The Calm Nights *Guide & Workbook*

Your companion to the Calm Nights Method — fourteen nights of helping your child learn to fall asleep, and fall back asleep, with you calm and close by.

This workbook belongs to _____ **Our child's name** _____

Age on night one _____ **Night one date** _____

Our starting point ☐ Stay Close ☐ Step Out & Check

STEPS 1–2 · BEFORE YOU BEGIN

How to use this workbook 2
Sleep basics at a glance 3
Is Calm Nights right for us? 4
Where we're starting 5

STEPS 3–4 · PREPARE & CHOOSE

Prep-week planner 6
Choosing our starting point 7
My plan at a glance 8
Our bedtime routine 9
Our team & hard-night plan 10

STEP 5 · THE 14 NIGHTS

Stay Close stage map 11
Step Out & Check log 12
Wake-ups, feeds & early mornings 13
Night-by-night journal 14
14-night overview 21

STEP 6 · REVIEW & BEYOND

Checkpoint reviews: nights 4, 7, 14 22
Troubleshooting & change log 25
After night 14 26
Cut-out cards 27

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

How to use this workbook

The programme teaches the method. This workbook is where you make it yours — and see your progress.

Your timeline

WHEN	WHAT YOU DO	WORKBOOK PAGES
Before prep week Steps 1–2 · Understand, check fit	Read steps 1–2 of the programme. Check fit and readiness. Log 3 ordinary nights so you know where you're starting from.	3, 4, 5
Prep week Steps 3–4 · Prepare, choose · 3–7 days	Room, feed, day, routine and team — one small job a day. Choose your starting point and fill in your plan.	6, 7, 8, 9, 10
Nights 1–14 Step 5 · Implement	Follow your starting point at bedtime, and the same response for every wake-up. Write up the night each morning — five minutes, over breakfast.	11–13, journal 14–20
Mornings after nights 4, 7, 14 Step 6 · Review	Checkpoint review: look at the numbers, tick what you see, decide: keep going, adjust, or get support.	21, 22–24
After night 14	Keep the foundations, plan for wobbles, and — if you need it — hand your notes to a 1:1 coach.	25, 26

Five rules that make the difference

1 · CONSISTENCY OVER PERFECTION
The same routine, the same words, the same response — every time, every adult, including at 3am. A good-enough plan done consistently beats a perfect plan done sometimes.

2 · DOWN CALM AND AWAKE
Drowsy is fine; asleep is not. Falling asleep where they'll wake up is what lets your child reconnect sleep cycles alone.

3 · PAUSE BEFORE YOU RESPOND
Many night noises are your child moving between cycles. Wait around five minutes, then use your starting point.

4 · ONE CHANGE AT A TIME
If you adjust anything, change one thing and give it three nights before judging it. Use the change log on page 25.

5 · WRITE IT DOWN, EVEN ON HARD NIGHTS
Progress hides at 2am. The numbers you write each morning are how you'll see it — and they're exactly what a coach would ask for if you ever need more help.

Symbols you'll use

✕ a wake-up you responded to · **O** a stir that settled on its own during your pause · **F** a feed (ml/oz or minutes) · **★** a win, however small · **1–10** peak crying level (scale on page 3) · ☐ tick when done

General guidance, not medical advice. Check with your pediatrician before you start, pause if your child becomes unwell, and always follow safe sleep guidance.

© Thyme4Sleep LLC · personal use only

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Sleep basics at a glance

The four ideas the whole programme rests on — and how to read your child's cry.

SLEEP PRESSURE
The longer we're awake, the stronger the drive to sleep. A fixed wake-up time and a well-timed bedtime make that drive peak when you need it.

SLEEP CYCLES
Every 45–90 minutes we all surface briefly. Adults roll over. Children who fell asleep fed, rocked or held look for that help again — that's the wake-up.

SLEEP ASSOCIATIONS
Whatever your child needs to *fall* asleep is usually what they need to fall *back* asleep. Change how bedtime ends and nights follow.

PREDICTABILITY FEELS SAFE
Children settle when they know what happens next. The same routine, words and response matter more than any single technique.

The crying scale

Once you know their needs are met — fed, clean, comfortable, well — use this to decide when to respond.

1	2	3	4	5	6	7	8	9	10

1–5 · Pause	Grizzling, whining, on-off fussing. They're working it out — give them the chance.
6–7 · Respond	Building, sustained crying. Respond with your starting point — the same way every time.
8–10 · Check & comfort	Distressed, frantic, or a different "something's wrong" cry. Go in, check, comfort until calm, then restart your starting point calmly.

Our child's sleep associations, right now

Circle everything your child currently needs to fall asleep at bedtime:

Breastfeed · Bottle · Rocking · Being held · Parent lying beside · Pacifier · Patting / shushing · Car / stroller · Hand-holding · TV or screen · Our bed · Other: _____

...and what they need when they wake in the night (if different):

THE ONE WE MOST WANT TO CHANGE

COMFORT WE'LL KEEP (CUDDLES, SONGS, LOVEY FROM 12M)

> **Remember:** sucking, rocking and cuddling to sleep aren't "bad". They only become a problem when they stop working for your family. The goal isn't less comfort — it's comfort that doesn't have to be repeated every 90 minutes.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Is Calm Nights right for us?

Be honest — starting before you're ready is the number one reason families give up on night three.

A good fit when...

- ☐ Our child is 6 months to around 4 years, healthy and growing well
- ☐ They usually need a feed, rocking, being held or us lying beside them to fall asleep — and wake looking for it
- ☐ We're ready to follow one approach consistently for 14 nights, without day-to-day support

Get personalised support first if...

- ☐ Your baby is under 6 months (or under 6 months adjusted age if born early)
- ☐ Reflux, allergies, snoring, pauses in breathing, weight concerns or ongoing pain
- ☐ Neurodivergence, additional needs or developmental delay
- ☐ Twins or multiples, or siblings sharing a room
- ☐ Naps and the daily schedule are the main problem

Ticked any of these? That's not a judgement — it means a general framework is likely to frustrate you. A plan built for your child will serve you better: thyme4sleep.com/pricing

READINESS · OUR CHILD

- ☐ Healthy right now — no illness, ear infection or new teeth cutting through
- ☐ Our pediatrician is happy with growth and with any changes to night feeds
- ☐ No big change in the next two weeks (travel, new sibling, daycare start)
- ☐ Sleeping in a safe space: alone, always started on their back, in a crib with only a fitted sheet

READINESS · OUR FAMILY

- ☐ Everyone who does bedtime has read steps 1–4
- ☐ We have two weeks of mostly home bedtimes
- ☐ We've agreed to try this together, the same way
- ☐ We've planned something kind for ourselves (early nights, help with the older kids)

Our decision

☐ **We're ready.** Prep week starts on _____ and night one is _____

☐ **Not quite yet.** We'll use the free Gentle Sleep Reset for a few days to stabilise, then revisit on _____

☐ **We need personalised support.** We'll look at 1:1 packages or email georgia@thyme4sleep.com

If you ever feel overwhelmed: lay your child down safely in their crib, step out, and give yourself a few minutes — a shower, music, a cup of tea, a call to a friend. Stepping away is a safe and responsible choice. If nothing soothes your child, contact your doctor. And if you've been feeling low or anxious for a while, please tell your own doctor too.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Where we're starting

Log three ordinary nights before prep week. Change nothing yet — just watch and write.

	BASELINE NIGHT A	BASELINE NIGHT B	BASELINE NIGHT C
Date			
Morning wake-up			
Naps (start–end)			
Routine started			
Lights out			
How they fell asleep			
Time to fall asleep			
Wake-ups (time · what we did)			
Up for the day			
Total night sleep (approx.)			

AVERAGE WAKE-UPS / NIGHT

AVERAGE TIME TO FALL ASLEEP

LONGEST STRETCH

What we hope looks different by night 14

Make it specific and realistic — e.g. "falls asleep in the crib in under 20 minutes", "one wake-up, settled in 10 minutes", "up after 6am".

Days shape nights. What happens in the day sets up the night. A consistent morning wake-up, plenty of daylight (especially in the morning), movement and floor play build healthy sleep pressure for bedtime. Too little daytime sleep can leave a child overtired and fighting sleep, while very late or very long naps can push bedtime later and lead to more wake-ups and early rising. Practising new skills during the day makes them less exciting at 2am. If your baseline shows naps or the daytime are the main problem, a 1:1 plan (which includes naps and schedule) will serve you better.

Naps are just for your records — Calm Nights doesn't include nap plans or daily schedules. These numbers are your "before". You'll compare against them at every checkpoint (pages 22–24).

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Prep-week planner

One small job a day. Have a shorter run-up? Combine days — but do them all before night one.

DAY · FOCUS	TICK AS YOU GO	DATE DONE
Day 1 **Read & decide**	☐ Every adult who does bedtime reads steps 1–4 of the programme ☐ Complete pages 4 and 5 of this workbook ☐ Pick a night-one date with two mostly-home weeks after it	
Day 2 **The room**	☐ Blackout curtains or window film up; test at dawn and at 7pm ☐ Mobiles and busy décor moved away from the crib ☐ Night light very dim and warm — or moved to the hallway	
Day 3 **The feed**	☐ Bedtime milk moved to the start of the routine, lights on ☐ Child stays awake for the whole feed ☐ Ask your pediatrician which night feeds (if any) to keep	
Day 4 **The day**	☐ Fixed morning wake-up time chosen (within 30 minutes) — and kept ☐ Daylight, movement and floor play every day ☐ Learning a new skill (rolling, sitting, crawling, standing)? Give lots of practice time during daytime wake windows, so it's less exciting to practise at bedtime and in the night	
Day 5 **The routine**	☐ New routine written on page 9 and started tonight ☐ Bedtime nudged earlier if it has crept late (15 minutes every couple of nights) ☐ Key phrase chosen and used at every bedtime from now on	
Day 6 **The team**	☐ Starting point chosen (page 7) and plan filled in (page 8) ☐ Who-does-which-night grid agreed (page 10) ☐ Hard-night plan agreed — so nobody debates it at 2am	
Day 7 **Ready**	☐ Script cards cut out and placed by the crib (page 27) ☐ From 12 months: tell your child what's changing with a simple story ☐ Something kind planned for yourselves this week	

OUR FIXED WAKE-UP TIME

_____ : _____ am

Within 30 minutes, every day — even after a bad night.

OUR TARGET LIGHTS-OUT

_____ : _____ pm

For most children, between 6:30 and 8:00pm.

Many families see improvement from preparation alone. If sleep gets noticeably better during prep week, wonderful — still start night one as planned. The foundations make the next 14 nights easier.

Night one date: _____

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Choosing our starting point

Both build the same skill and both keep you present. The difference is how much of you is in the room.

STARTING POINT A

Stay Close

You stay beside the crib with touch and voice, then move a little further away every few nights until you're out of the room.

STARTING POINT B

Step Out & Check

Key phrase, then you leave — returning for brief, calm, identical checks if crying builds.

Answer three questions — circle one answer each

QUESTION	POINTS TO STAY CLOSE	POINTS TO STEP OUT & CHECK
1. When you stay in the room at bedtime, does your child generally calm down or wind up?	Calms with us nearby · Not sure	Gets more worked up, plays or chats
2. Which could you repeat at 2am, every night for two weeks? *(this one wins a tie)*	Sitting quietly beside the crib through some protest	Stepping out and returning for short checks
3. Can every adult who does bedtime do it the same way?	Yes → keep your choice. No → choose the one everyone can do. Consistency between adults matters more than fit.	

PARENT / CARER 1 WOULD CHOOSE

☐ Stay Close ☐ Step Out & Check because...

PARENT / CARER 2 WOULD CHOOSE

☐ Stay Close ☐ Step Out & Check because...

Still unsure? Start with Stay Close — it's the gentlest, and easy to move on from. If neither feels possible for your family, that's a sign to get personalised support rather than force a fit.

OUR COMMITMENT

We are choosing ☐ **Stay Close** ☐ **Step Out & Check**. We'll give it **7 full nights** before considering a switch. Night 7 is our only switch point, and we switch once. We'll use the same words and the same response every time — including at 3am.

Signed

Signed

The one you can repeat beats the "right" one you can't.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

My plan at a glance

Fill this in together. Every adult who does bedtime follows the same page.

OUR PLAN	OUR ANSWER
Our starting point	☐ Stay Close ☐ Step Out & Check
Why we chose it	
Our key phrase	
Morning wake-up time	
Routine starts / lights out	
What we'll do instead of the old support	
Night feed our doctor wants us to keep Time, amount — or "none"	
Comfort feeds we'll reduce See page 13	
Our pause before responding	About _____ minutes (at least 5)
What counts as morning	Not before 6:00am. Content between 6–7am? We may wait until _____
Who does which nights Full grid on page 10	
Our hard-night plan What we'll do at 2am when we want to give up	

Our three non-negotiables

The things we will do the same way every single time (e.g. "down awake", "no picking up during checks", "key phrase at every response").

1

2

3

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Our bedtime routine

20–40 minutes. Same steps, same order, same calm energy. Not long — predictable.

#	THE SUGGESTED STEP	OUR VERSION (WHAT, WHERE)	TIME	WHO
1	**Feed (lights on)** Milk at the start, outside the bedroom where possible. Fully awake.			
2	**Bath or wash** Lukewarm, a gentle soak — a strong wind-down cue.			
3	**PJs, sleep sack, teeth** Into the dim bedroom from here. Screens off.			
4	**Books & song** Two books, one song. Dim light. Lots of cuddles.			
5	**Key phrase** Into the crib calm and awake. Say your phrase.			
6	**Your starting point** Stay Close or Step Out & Check — the same every time.			

Our key phrase

Short, warm, identical every time. Tick one, or write your own. Every adult uses the exact same words.

- ☐ "It's sleepy time now. I love you very much. See you in the morning."
- ☐ "Night night, my love. Time for sleep. I'll see you when the sun comes up."
- ☐ "You're safe, you're loved, and it's time to rest. Goodnight."
- ☐ Our own: _____

FROM 12 MONTHS · OUR "WHAT'S CHANGING" STORY

A short, simple story about your child, told in the day and at bedtime for a few days before night one. E.g. "When it's sleepy time, [name] has milk, a bath and two books. Then Mummy says our special words and [name] snuggles down in the cot all by themselves. Daddy is close by..."

_____

_____

_____

ROUTINE HELPERS

- ☐ Feed is fully awake, lights on
- ☐ Screens off from step 3
- ☐ Room dark by the books
- ☐ White noise on (optional)
- ☐ Sleep sack / lovey ready (lovey from 12m)
- ☐ Toddlers: bedtime chart & Bedtime Pass
- ☐ Choices offered *before* bed, not after

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Our team & hard-night plan

Decide it now, in daylight, so nobody has to decide it at 2am.

Who leads each night

	1	2	3	4	5	6	7	8	9	10	11	12	13	14
Bedtime														
Wake-ups														
Morning														

Write initials. If one adult finds settling harder, they take a different role (bath and books) while the other leads.

If… then…

If it's 2am and we want to give up…	Swap, step out for 5 minutes, re-read the script card. We don't change the plan at night.
If crying reaches 8–10…	Go in, check, comfort until calm, then restart our starting point.
If our child seems unwell…	Pause the plan, comfort fully, follow medical advice. Restart at the same stage once well.
If we disagree in the moment…	Follow this page now; talk it through the next morning with the journal in front of us.
If one of us is running on empty…	
If _____	

LOOKING AFTER OURSELVES THESE TWO WEEKS

Early nights, a nap swap, meals sorted, help with older children…

OUR PEOPLE

Someone to text on a hard night

Pediatrician phone

Unity matters more than which starting point you chose.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Stay Close stage map

Using Stay Close? This is your route out of the room. Using Step Out & Check? Skip to page 12.

NIGHTS	WHERE YOU ARE	WHAT YOU DO	DONE	HOW IT WENT
Nights 1–3	**Beside the crib** ● ○ ○ ○ ○	Sit or kneel right next to the crib. Offer touch — a hand on the tummy, stroking the back or head — and soft shushing. Reduce touch as they calm; stop before they're asleep if you can. No picking up.		
Nights 4–6	**Beside the crib, voice only** ● ● ○ ○ ○	Same spot, but hands off. Comfort with your voice and presence. If crying builds past a 6 out of 10, a brief touch is fine — then back to voice.		
Nights 7–9	**Halfway to the door** ● ● ● ○ ○	Move your chair halfway across the room. Calm, low, repetitive reassurance (your key phrase, a gentle "shhh"). Keep eye contact soft and minimal.		
Nights 10–12	**In the doorway** ● ● ● ● ○	Sit in the open doorway where they can see or hear you. Reassure with your key phrase at intervals rather than constantly.		
Nights 13–14	**Just outside the door** ● ● ● ● ●	Out of sight, within earshot. Offer your key phrase from outside if needed.		

The Stay Close rules

MOVE ON SCHEDULE
Even if the previous stage was going well. Lingering is how your presence becomes the new support.

NO PICKING UP OR ROCKING
It sends a mixed message. If they stand, lay them down once, then let them sit or stand while you stay calm.

BORING IS YOUR FRIEND
Low voice, soft eyes, minimal chat. You're a calm, quiet presence — not entertainment.

WAKE-UPS MATCH BEDTIME
Respond from the stage you're at that night — same position, same words.

Our room map

Sketch your child's room and mark where your chair goes at each stage (1 = beside the crib … 5 = outside the door).

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Step Out & Check log

Using Step Out & Check? Log your checks for the first three bedtimes — you'll see pauses get easier.

THE SCRIPT

1 Key phrase & leave, warm and confident. **2** Pause at least 5 minutes — check the clock. **3** Crying building (6+)? Go in briefly: all is well, a quick stroke, no picking up. **4** Key phrase & leave again — under a minute. **5** Repeat, keeping each pause at least as long as the last.

NIGHT 1 · BEDTIME · DATE _____

CHECK	WENT IN AT	PAUSED (MIN)	CRY 1–10	MINS IN ROOM	NOTES
1					
2					
3					
4					

Asleep at _____ · total time to fall asleep _____ min

NIGHT 2 · BEDTIME · DATE _____

CHECK	WENT IN AT	PAUSED (MIN)	CRY 1–10	MINS IN ROOM	NOTES
1					
2					
3					
4					

Asleep at _____ · total time to fall asleep _____ min

NIGHT 3 · BEDTIME · DATE _____

CHECK	WENT IN AT	PAUSED (MIN)	CRY 1–10	MINS IN ROOM	NOTES
1					
2					
3					
4					

Asleep at _____ · total time to fall asleep _____ min

Checks making crying louder? Lengthen the pause and keep every check identical: same words, same touch, same exit. Many children fall asleep around the 15–20 minute mark.

Five minutes at bedtime feels like twenty. Use the clock.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Wake-ups, feeds & early mornings

The same calm response at 2am as at bedtime. Here's the whole night on one page.

When you hear them

A noise from the crib

↓

Pause ~5 minutes. Many noises are cycle changes and settle alone (mark O).

↓ still crying, 6+

Is it before 6am? Yes → it's night. Use *exactly* your bedtime starting point and key phrase.

↓

Is it a feed your doctor wants you to keep? Feed calmly, lights low, then straight back into the crib with your key phrase.

↓ crying 8–10 or a different cry

Check & comfort until calm, then restart your starting point.

The 6am rule

- Before 6am is night: same pause, same response.
- Between 6 and 7am, if they're content, you can leave them until a more family-friendly hour.
- Still asleep 12–12½ hours after bedtime? Gently wake them.

The boring morning

- Curtains open, cheerful "good morning".
- Boring things first: diaper, dressed, teeth.
- First milk *outside* the bedroom — then the cuddles.
- From about two, a toddler clock helps.

Comfort-feed reduction tracker

Only for feeds your pediatrician is happy to drop. Reduce gradually: a couple of minutes off a breastfeed, or around 30ml (1oz) off a bottle, every night or two.

NIGHT	1	2	3	4	5	6	7	8	9	10	11	12	13	14
Feed 1 · time														
ml / oz / min														
Feed 2 · time														
ml / oz / min														

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Nights 1 & 2

Five minutes each morning. Write it up even — especially — after a hard night.

Night 1

Date _____ Led by _____

Stay Close stage: **Beside the crib**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Night 2

Date _____ Led by _____

Stay Close stage: **Beside the crib**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Any progress, however small, is a win.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Nights 3 & 4

Five minutes each morning. Write it up even — especially — after a hard night.

Night 3

Date _____ Led by _____

Stay Close stage: **Beside the crib**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Night 4

Date _____ Led by _____

Stay Close stage: **Beside the crib, voice only**
★ **Checkpoint review tomorrow — page 22**

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Night 3 feeling worse is common. Hold steady.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Nights 5 & 6

Five minutes each morning. Write it up even — especially — after a hard night.

Night 5 Date _____ Led by _____

Stay Close stage: **Beside the crib, voice only**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	✕ / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Night 6 Date _____ Led by _____

Stay Close stage: **Beside the crib, voice only**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	✕ / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Any progress, however small, is a win.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Nights 7 & 8

Five minutes each morning. Write it up even — especially — after a hard night.

Night 7 Date _____ Led by _____

Stay Close stage: **Halfway to the door**
★ **Checkpoint review tomorrow — page 23**

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Night 8 Date _____ Led by _____

Stay Close stage: **Halfway to the door**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Any progress, however small, is a win.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Nights 9 & 10

Five minutes each morning. Write it up even — especially — after a hard night.

Night 9 Date _____ Led by _____

Stay Close stage: **Halfway to the door**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Night 10 Date _____ Led by _____

Stay Close stage: **In the doorway**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Any progress, however small, is a win.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Nights 11 & 12

Five minutes each morning. Write it up even — especially — after a hard night.

Night 11 Date _____ Led by _____

Stay Close stage: **In the doorway**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Night 12 Date _____ Led by _____

Stay Close stage: **In the doorway**
Step Out & Check: same script as night 1

BEDTIME

Routine start	Lights out	Asleep by	Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	× / O / F

Up for the day	Longest stretch	★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Any progress, however small, is a win.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Nights 13 & 14

Five minutes each morning. Write it up even — especially — after a hard night.

Night 13 Date _____ Led by _____

Stay Close stage: **Just outside the door**
Step Out & Check: same script as night 1

BEDTIME

Routine start | Lights out | Asleep by | Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	✕ / O / F

Up for the day | Longest stretch | ★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Night 14 Date _____ Led by _____

Stay Close stage: **Just outside the door**
★ **Checkpoint review tomorrow — page 24**

BEDTIME

Routine start | Lights out | Asleep by | Checks / returns (#)

Peak crying 1 2 3 4 5 6 7 8 9 10 · ☐ Down awake ☐ Key phrase used ☐ Feed at start of routine

THROUGH THE NIGHT

TIME	HOW LONG	PEAK 1–10	WHAT WE DID	✕ / O / F

Up for the day | Longest stretch | ★ Win of the night

How consistent were we? ☐ Every response matched bedtime ☐ Same words every time ☐ We followed the hard-night plan
Notes: _____

Any progress, however small, is a win.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

14-night overview

Copy the key numbers here from each journal page. At a glance, this is where progress shows.

NIGHT	LIGHTS OUT	MINS TO SLEEP	PEAK CRY	WAKE-UPS (#)	LONGEST STRETCH	UP FOR DAY	WIN OF THE NIGHT
Base							From page 5
1							
2							
3							
4 review							
5							
6							
7 review							
8							
9							
10							
11							
12							
13							
14 review							

MINS TO SLEEP: NIGHT 1 → 14

WAKE-UPS: NIGHT 1 → 14

LONGEST STRETCH: NIGHT 1 → 14

Shaded rows are your checkpoint nights — turn to pages 22–24 the next morning. Progress is rarely a straight line: look at the trend across three or four nights, not one.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Night 4 — Is it moving?

The morning after night 4. Ten quiet minutes with your journal and overview in front of you.

The numbers

	BASELINE (PAGE 5)	NIGHT 1	NIGHT 3	NIGHT 4
Minutes to fall asleep				
Peak crying at bedtime				
Number of wake-ups				
Longest stretch				
Up for the day				

What we're seeing

- [ ] Lay-down is a little calmer
- [ ] Falling asleep takes less time than night 1
- [ ] At least one wake-up was shorter or quieter
- [ ] Something else that's better: _____

How consistent have we been?

- [ ] Every bedtime used the same routine, starting point and key phrase
- [ ] Every wake-up got the same response as bedtime — including at 3am
- [ ] Morning wake-up time stayed within 30 minutes
- [ ] We ran the "Check the basics" list on page 25

Our decision — circle one

KEEP GOING
Any one of these is progress. Keep going exactly as you are — no changes.

ADJUST
None yet, or night 3 felt worse? That's common. Run the "Check the basics" list in Troubleshooting, change nothing else, and hold steady to night 7.

GET SUPPORT
Crying is escalating to distress (8–10) for long stretches every night, or your child seems unwell: pause, speak to your pediatrician, and consider personalised support.

Our one action before tonight

You'll also receive a check-in email for this checkpoint — the buttons in it take you to the matching next step. "Get support" is a decision, not a failure.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Night 7 — Is the skill landing?

The morning after night 7. Ten quiet minutes with your journal and overview in front of you.

The numbers

	BASELINE (PAGE 5)	NIGHT 1	NIGHT 6	NIGHT 7
Minutes to fall asleep				
Peak crying at bedtime				
Number of wake-ups				
Longest stretch				
Up for the day				

What we're seeing

- ☐ Most bedtimes, they fall asleep with your starting point, not the old support
- ☐ Fewer wake-ups, or shorter ones
- ☐ You feel clearer about what to do at 2am
- ☐ Something else that's better: _____

How consistent have we been?

- ☐ Every bedtime used the same routine, starting point and key phrase
- ☐ Every wake-up got the same response as bedtime — including at 3am
- ☐ Morning wake-up time stayed within 30 minutes
- ☐ Stay Close: we moved stage on schedule

Our decision — circle one

KEEP GOING
Keep going to night 14. If you're on Stay Close, keep moving on schedule.

ADJUST
Bedtime is better but nights aren't: check every wake-up gets the same response as bedtime. Use the Troubleshooting library for the specific sticking point.

GET SUPPORT
Little or no change despite following the plan every time (including 3am)? That's not a failure — it's information that your child needs a plan built around them.

Switch point: night 7 is the only time to consider switching starting point — and you switch once. ☐ We're staying with our choice ☐ We're switching to _____ because _____

Our one action before tonight

You'll also receive a check-in email for this checkpoint — the buttons in it take you to the matching next step. "Get support" is a decision, not a failure.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Night 14 — Where have we landed?

The morning after night 14. Ten quiet minutes with your journal and overview in front of you.

The numbers

	BASELINE (PAGE 5)	NIGHT 1	NIGHT 13	NIGHT 14
Minutes to fall asleep				
Peak crying at bedtime				
Number of wake-ups				
Longest stretch				
Up for the day				

What we're seeing

- ☐ They fall asleep on their own most nights
- ☐ Wake-ups resettle with a brief, calm response — or don't happen
- ☐ Mornings start at 6am or later
- ☐ Something else that's better: _____

How consistent have we been?

- ☐ Every bedtime used the same routine, starting point and key phrase
- ☐ Every wake-up got the same response as bedtime — including at 3am
- ☐ Morning wake-up time stayed within 30 minutes
- ☐ We gave any adjustment at least three nights before judging it

Our decision — circle one

KEEP GOING
Wonderful. Move to "Keeping it going" in the plan.

ADJUST
Mostly there? Hold the same plan for another 7 nights before changing anything. Progress often finishes quietly.

GET SUPPORT
Still waking frequently, splitting nights, early rising before 5:30am, or nap problems undoing nights? This is where personalised assessment adds what a general plan can't.

Our one action before tonight

You'll also receive a check-in email for this checkpoint — the buttons in it take you to the matching next step. "Get support" is a decision, not a failure.

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Troubleshooting & change log

General answers to the most common sticking points. Change one thing at a time; give it three nights.

STICKING POINT	WHAT TO TRY
Check the basics (do this first)	Is the milk feed truly at the start of the routine, with books or bath after it? Is bedtime well-timed (not overtired)? Is the morning wake-up time consistent? Is every adult using the same starting point and key phrase — including at 3am? Most stalls come from one of these.
“Night 3 got worse!”	Very common, and usually a sign your child is testing whether the new pattern is real. Hold steady to your night 4 checkpoint. Families who change course here start again from zero.
Illness or teething	Pause, comfort as much as they need, and follow medical advice. Keep the routine and wake-up time. When they’re well (usually 2–3 days after symptoms ease), restart at the stage you’d reached — it comes back quickly.
They stand up and won’t lie down	Practise “up and down” lots during the day. At night, lay them down once, then leave them to it — they will lie down when tired. Repeatedly laying them down becomes a game.
Vomiting from crying	Clean up calmly and quickly with low light and little interaction, change the sheet, then continue your starting point. If it happens repeatedly, pause and speak to your pediatrician.
Early rising (before 6am)	Treat it as night: same pause, same response. Then look at the day: is bedtime too late? Is the room light at dawn? Is there an early-morning feed still being offered?
Travel & time zones	Bring the routine, sleep sack, white noise and key phrase. Shift to local time straight away with daylight and mealtimes. Expect a few unsettled nights, then return to your starting point.
The toddler negotiator	Use the bedtime chart and Bedtime Pass cards. Offer choices before bed (“blue PJs or stars?”), not after. Once in bed: key phrase, same response, no new debates.
We disagree as parents	Agree the starting point and the hard-night plan before night one. If one parent finds settling harder, they take a different role (bath and books) while the other leads. Unity matters more than which starting point you chose.
Our checkpoint says “get support”	Then that’s the right next step, not a failure. See “Keeping it going” below — Georgia will build on what you’ve already done.

Our change log

DATE	THE ONE THING WE CHANGED	AFTER 3 NIGHTS…	KEEP?
			Y / N
			Y / N
			Y / N
			Y / N
			Y / N

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

After night 14

Sleep isn't about perfection — it's about consistency, intention and loving boundaries.

KEEP THESE FOUNDATIONS

- [ ] Same routine and key phrase
- [ ] Down calm and awake
- [ ] Pause before responding
- [ ] Fixed morning wake-up
- [ ] The 6am rule and the boring morning

OUR WOBBLE PLAN

Colds, teeth, travel, a new sibling. Comfort fully, keep the routine and wake-up time, then:

- [ ] Steady things with the free Gentle Sleep Reset
- [ ] Restart our starting point at the stage we'd reached
- [ ] Re-read this workbook's page 8 together

Looking back

The biggest change we've seen

What helped most

What was hardest

A moment we're proud of

If you'd like personalised support

Get support if any of these apply: no meaningful change by night 7, despite following the plan every time; crying escalates night after night instead of easing; still waking frequently, splitting nights or rising before 5:30am at night 14; you and your partner can't agree on or sustain an approach. A Starter package gives you a plan written for your child, including naps and schedule; Standard adds a consultation call and two weeks of support.

HANDOVER SUMMARY · BRING THIS TO YOUR 1:1

Starting point & stage reached

Night 1 vs night 14 (sleep, wake-ups)

Where we're stuck

Naps & daytime right now

What we'd most like help with

Tell Georgia you've done Calm Nights and share this page with your journal — she'll build on what you've learned rather than start from scratch. thyme4sleep.com/pricing · georgia@thyme4sleep.com

You are doing something truly wonderful for your child — and for yourselves. — Georgia

© Thyme4Sleep LLC · personal use only

THYME 4 SLEEP
PEDIATRIC SLEEP COACHING

Cut-out script cards

Cut along the dashed lines. Keep the one you need by the crib — and a copy on the fridge.

✂ OUR KEY PHRASE

_____

_____

_____

Short, warm, identical every time. Every adult. Every response.

Wake-up: _____ : _____ am **Lights out:** _____ : _____ pm

Hard-night plan:

_____

✂ STAY CLOSE

- **Nights 1–3:** Beside the crib
- **Nights 4–6:** Beside the crib, voice only
- **Nights 7–9:** Halfway to the door
- **Nights 10–12:** In the doorway
- **Nights 13–14:** Just outside the door

Move on schedule. No picking up. Low voice, soft eyes. Wake-ups get the same response from tonight's stage.

✂ STEP OUT & CHECK

1. Crib, calm and awake. Key phrase. Leave.
2. Pause at least 5 minutes — use the clock.
3. Crying 6+? In briefly. Quick stroke, no picking up.
4. Key phrase. Out in under a minute.
5. Repeat. Each pause at least as long as the last.

Every check identical: same words, same touch, same exit.

✂ AT 2AM

Pause ~5 min. Before 6am = night.

1–5 pause · **6–7** respond with our starting point · **8–10** check & comfort, then restart calmly.

Feed we're keeping: _____ — lights low, straight back down.

Overwhelmed? Lay them down safely, step out, breathe. Swap with your partner.

Comfort is never removed. Confusion is.

© Thyme4Sleep LLC · personal use only